

HEALTHCARE ANALYTICS PROJECT

Vitalis

Healthcare data, transformed into decisions.

Six Power BI dashboard pages, a corrected SQL workbook, and a React web app — all built on a synthetic patient dataset of 20,000 appointments across 5 clinics.

PROJECT DOCUMENTATION



What this project is

Vitalis is a healthcare analytics project that analyses patient, financial, and operational data using Power BI dashboards. The objective is to understand patient demographics, service utilisation, financial performance, and operational efficiency across multiple clinic locations and medical specialties.

The project transforms raw healthcare data into meaningful insights that can support better decision-making and improved healthcare service delivery.

AT A GLANCE

\$6M Total revenue

5,000 Patients

20,000 Appointments

15,000 Diagnoses

5 Clinics

What the dashboards cover

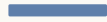
Five comprehensive views built on the same underlying data — each one designed to answer a different question about the practice.



FINANCIAL

Money

Total revenue (~\$6M), patient payments (~\$3M), revenue by location, by specialty, by age group.



GEOGRAPHIC

Place

Patient distribution across Seattle, Spokane, Tacoma, Portland, and Olympia.



CLINICAL

Specialty

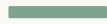
Appointment activity and revenue by Orthopedics, Cardiology, Oncology, Neurology, Pediatrics, General Medicine.



DEMOGRAPHIC

People

Gender, age group, insurance provider, and city-based distributions of the patient base.



OPERATIONAL

Activity

Appointments by status (Completed / Cancelled / No-show), by month, by day of week, and diagnosis counts.

Six tables, one fact table

Appointments is the central fact table. Billing, diagnoses, and medications all reference it via foreign keys.

patients

patient_id
first_name
last_name
gender • dob
city • insurance

billing

bill_id
appointment_id ↗
amount
insurance_covered
patient_paid

appointments

appointment_id
patient_id ↗
doctor_id ↗
date • status
visit_reason

doctors

doctor_id
doctor_name
specialty
clinic_location

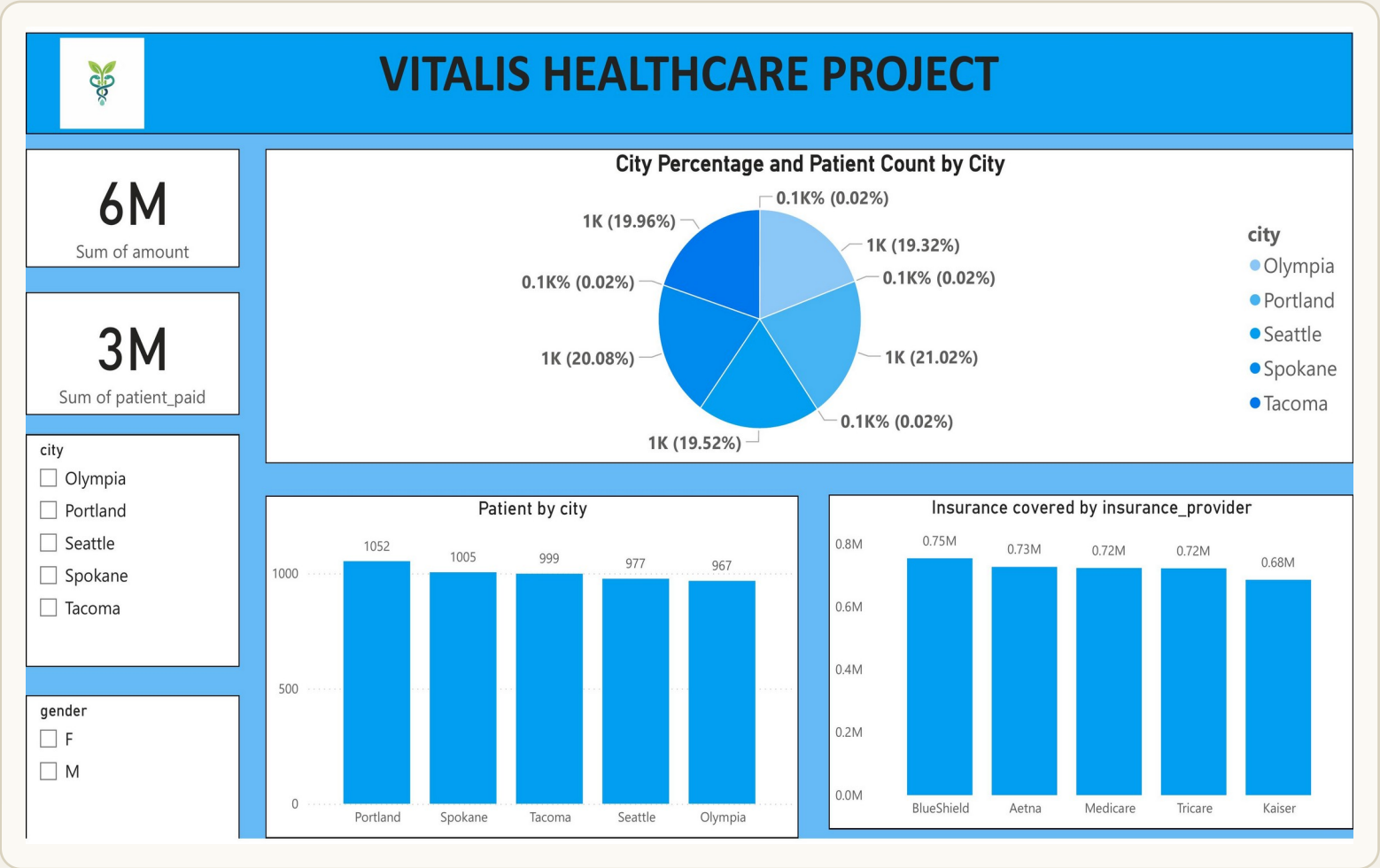
medications

med_id
patient_id ↗
medication_name
dosage
start_date • end_date

diagnoses

diagnosis_id
appointment_id ↗
diagnosis_code
diagnosis_description

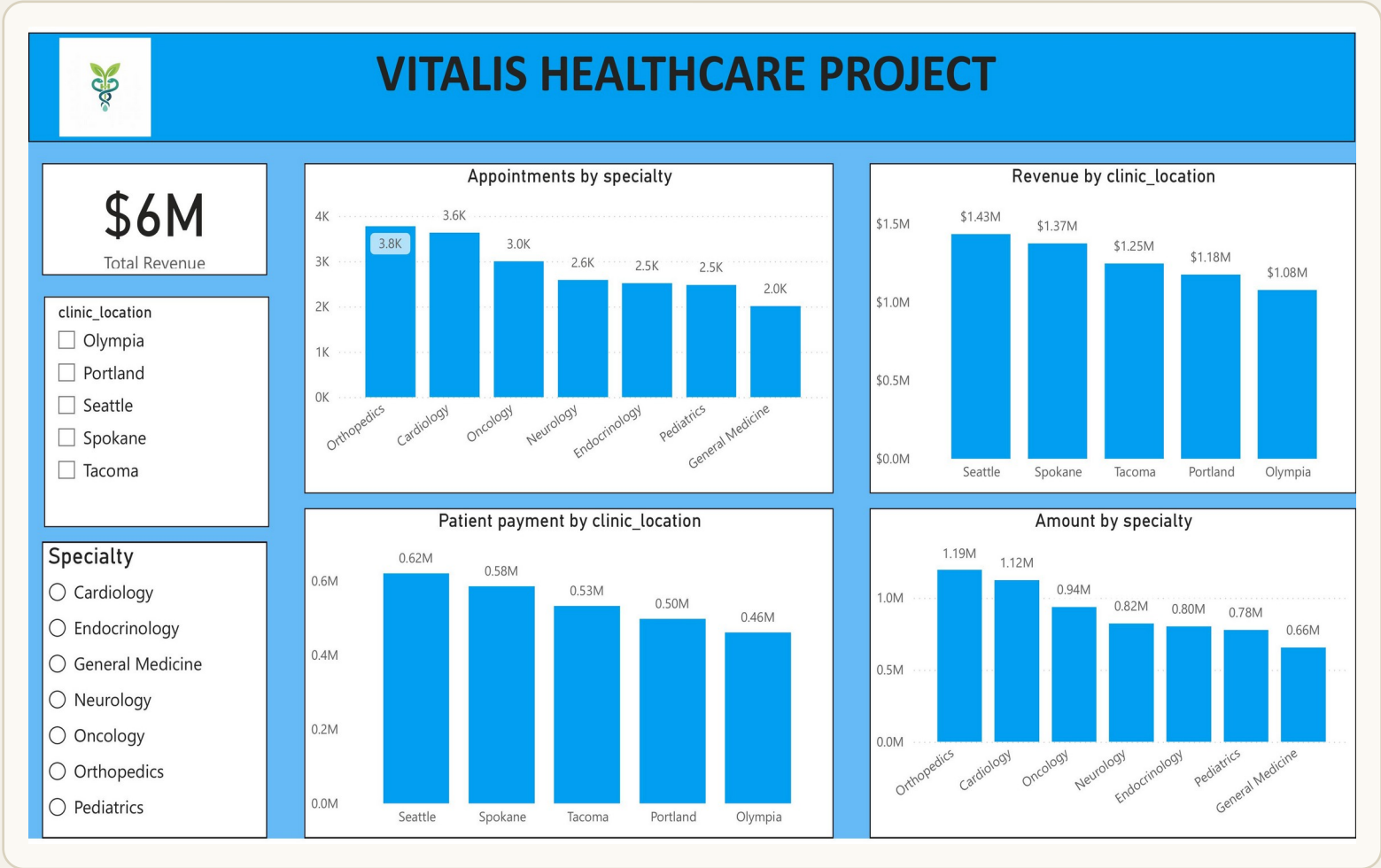
Who the patients are



KEY OBSERVATIONS

- 15,000 patients across 5 cities — almost evenly distributed
- 2Portland leads with 1,052 patients; Olympia trails at 967
- 3Insurance coverage is well-spread: BlueShield (\$0.75M) just edges Aetna (\$0.73M)
- 4Gender split is balanced (F / M) across every city

Where the revenue comes from



KEY OBSERVATIONS

- 1

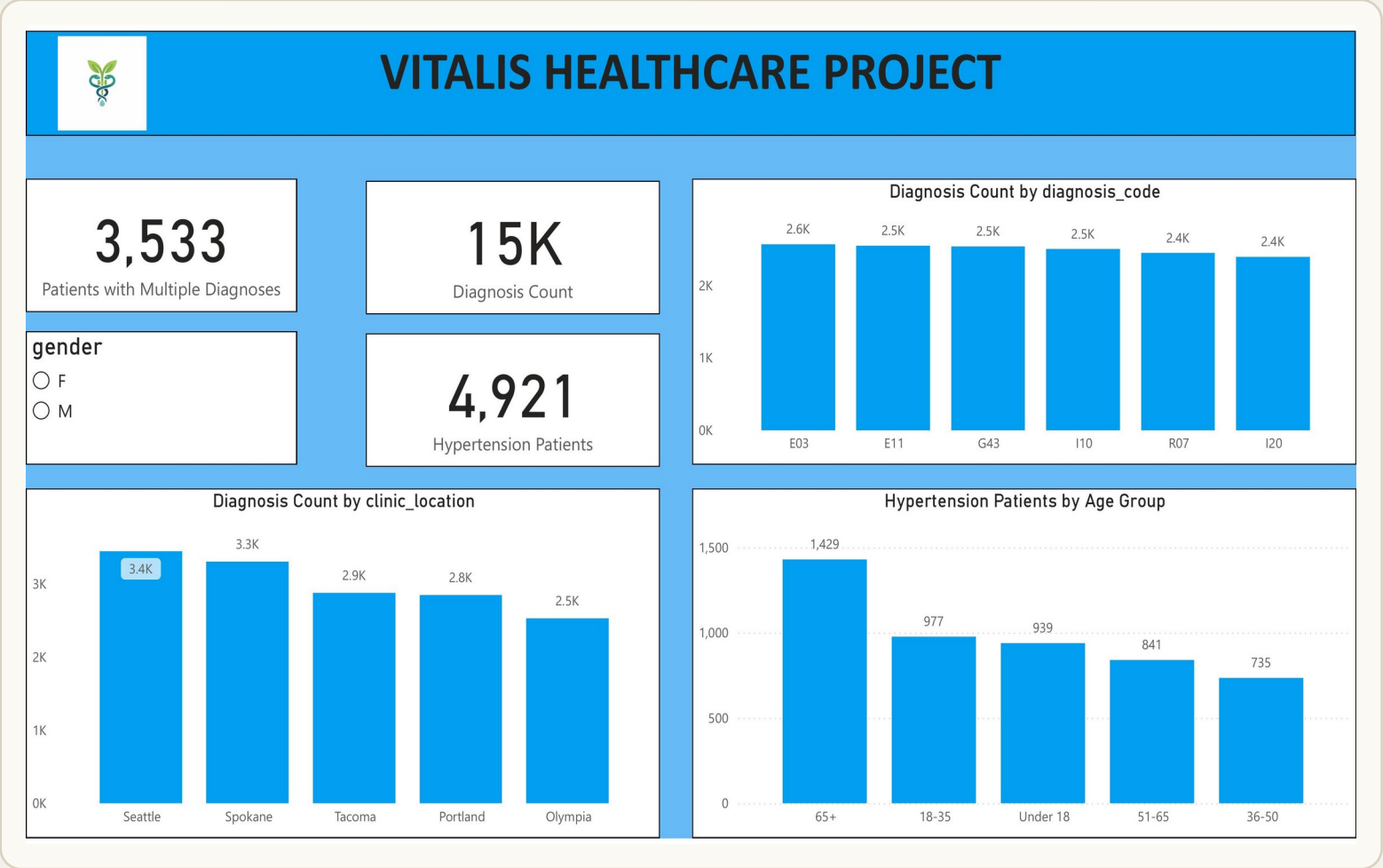
Total revenue: \$6M across all specialties and clinics
- 2

Orthopedics is the top specialty by revenue (\$1.19M) and volume (3.8K appointments)
- 3

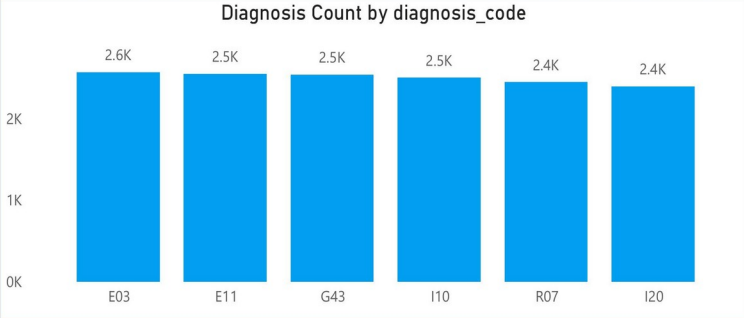
Seattle leads clinic revenue at \$1.43M; Olympia trails at \$1.08M
- 4

General Medicine is lowest-grossing at \$0.66M

What the patients have

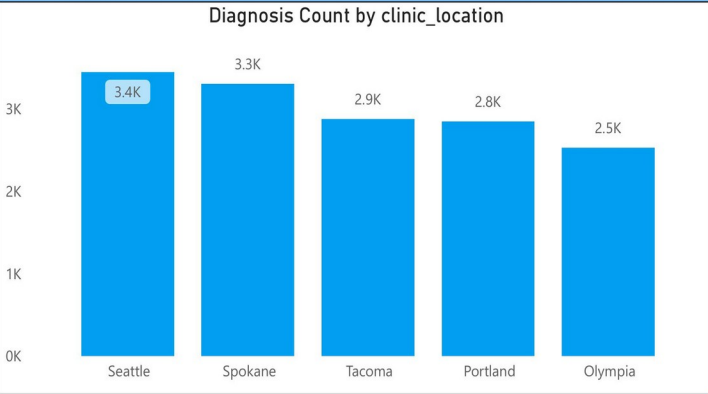


Diagnosis Count by diagnosis_code



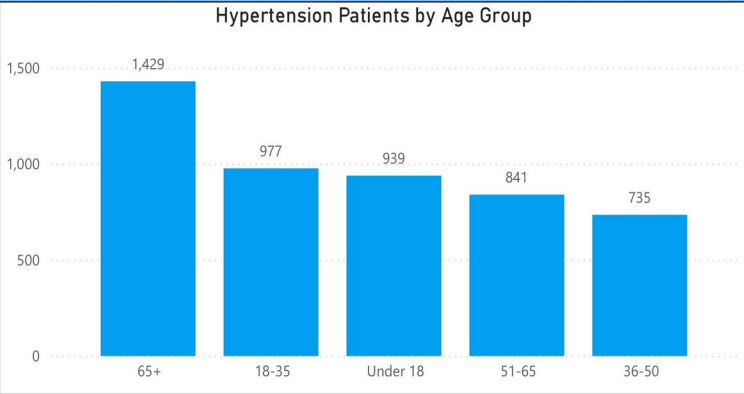
diagnosis_code	Count
E03	2,600
E11	2,500
G43	2,500
I10	2,500
R07	2,400
I20	2,400

Diagnosis Count by clinic_location



clinic_location	Count
Seattle	3,400
Spokane	3,300
Tacoma	2,900
Portland	2,800
Olympia	2,500

Hypertension Patients by Age Group



Age Group	Count
65+	1,429
18-35	977
Under 18	939
51-65	841
36-50	735

KEY OBSERVATIONS

- 1

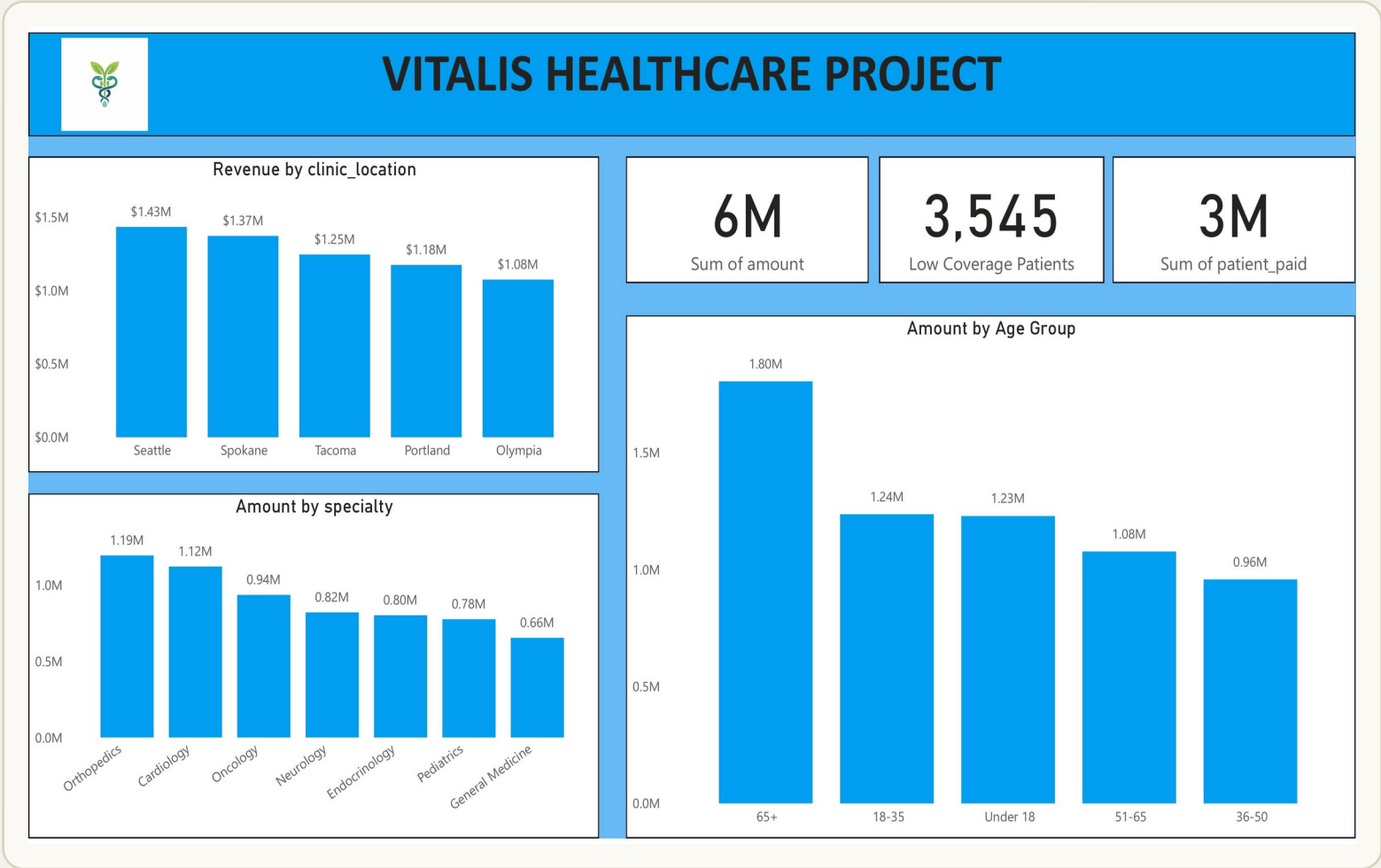
15,000 diagnoses recorded across the patient population
- 2

4,921 patients carry a hypertension diagnosis (I10)
- 3

3,533 patients have multiple concurrent diagnoses
- 4

Diagnosis counts are roughly even across codes — no single condition dominates

How the money flows



KEY OBSERVATIONS

- 1

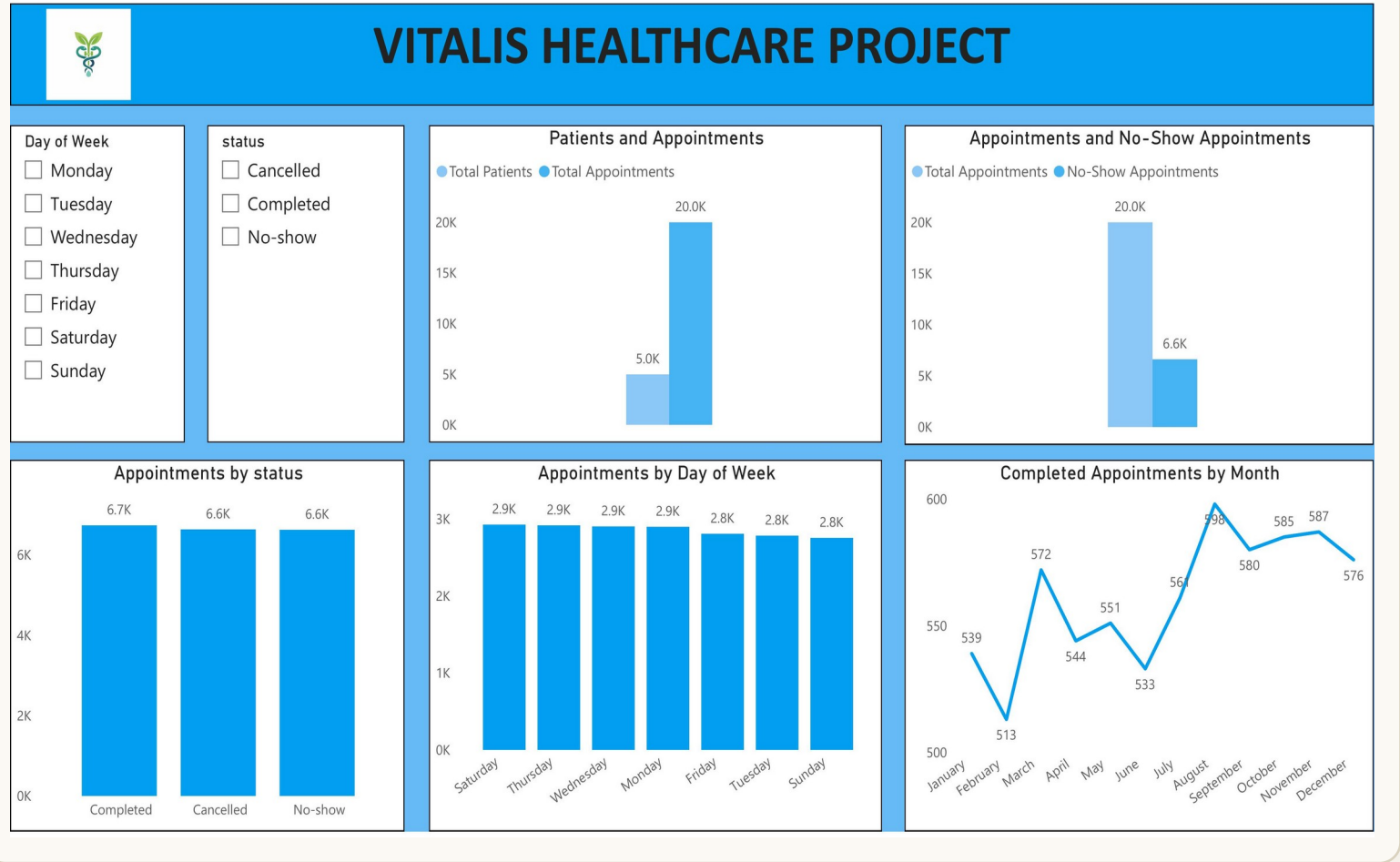
\$6M total billed • \$3M paid by patients • 3,545 patients with low coverage
- 2

Age 65+ generates the highest billed amount: \$1.80M
- 3

Orthopedics + Cardiology together drive \$2.31M (39% of revenue)
- 4

Seattle clinic outperforms Olympia by 32% in revenue

How the practice runs



Patients and Appointments

Total Patients

Total Appointments

Day	Total Patients	Total Appointments
Monday	5.0K	20.0K

Appointments and No-Show Appointments

Total Appointments

No-Show Appointments

Day	Total Appointments	No-Show Appointments
Monday	20.0K	6.6K

Appointments by status

Status	Count
Completed	6.7K
Cancelled	6.6K
No-show	6.6K

Appointments by Day of Week

Day	Count
Saturday	2.9K
Thursday	2.9K
Wednesday	2.9K
Monday	2.9K
Friday	2.8K
Tuesday	2.8K
Sunday	2.8K

Completed Appointments by Month

Month	Count
January	539
February	513
March	572
April	544
May	551
June	533
July	564
August	598
September	580
October	585
November	587
December	576

KEY OBSERVATIONS

- 1

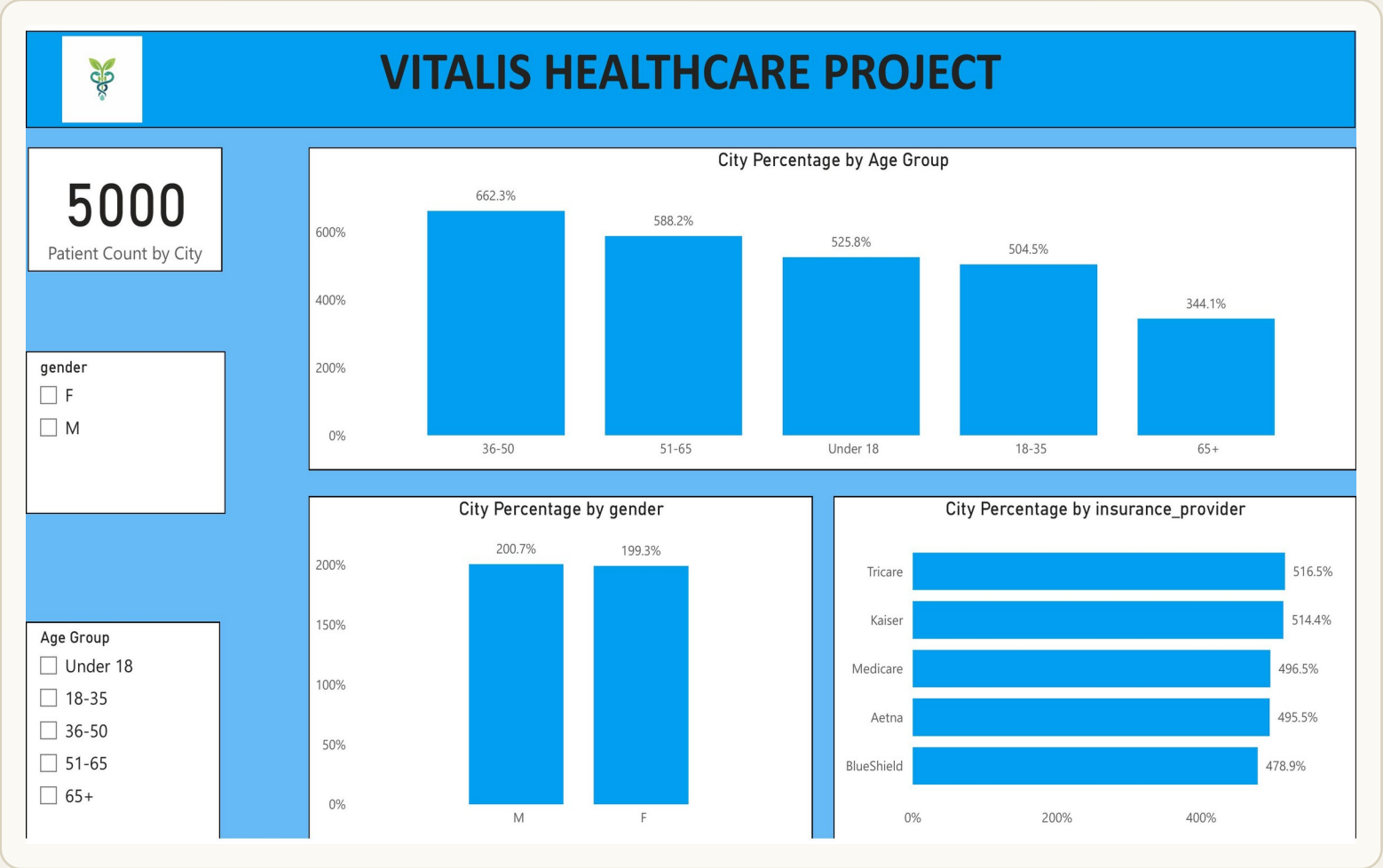
20,000 total appointments • 6.6K no-shows (33% — major operational drag)
- 2

Appointment volume is flat across weekdays (2.8K–2.9K) — no peak day
- 3

Completed appointments peaked in August at 598
- 4

Status distribution is nearly even: 6.7K completed / 6.6K cancelled / 6.6K no-show

Population distribution



KEY OBSERVATIONS

- 1

All 5,000 patients accounted for across cities
- 2

Age 36-50 is the largest cohort by city share
- 3

Gender distribution within cities is near 50/50 (M 200.7% vs F 199.3% summed)
- 4

Insurance providers split evenly — Tricare and Kaiser slightly ahead

What the data tells us

—
\$1.43M

Seattle leads

top-grossing clinic location

—
33%

No-show rate

~6.6K missed appointments — costly

—
\$1.19M

Orthopedics tops

highest-revenue specialty

—
4,921

Hypertension patients

98% of the active patient base

—
\$1.80M

Age 65+ revenue

highest-spending age cohort

—
3,533

Multi-diagnosis pts.

carry more than one chronic condition

What to do about it

Five concrete actions, ranked by expected impact.

- 01 Reduce no-show rates**
Implement automated SMS/email reminders and easier rescheduling. Industry data suggests this cuts no-shows by ~30%, recovering significant capacity.
- 02 Strengthen chronic disease management**
Develop targeted programs for hypertension and diabetes. 3,533 patients carry multiple diagnoses — coordinated care reduces complications and repeat visits.
- 03 Optimise high-revenue specialties**
Invest in Orthopedics and Cardiology (the top two revenue producers): specialist staffing, equipment upgrades, throughput improvements.
- 04 Improve lower-revenue locations**
Audit Olympia and Portland (lowest-grossing clinics): access, staffing, specialty mix. Localised improvement plans can close the gap with Seattle.
- 05 Enhance preventive care for 65+**
Patients 65+ generate the highest treatment costs (\$1.80M). Expand screening and preventive programs to manage long-term cost and improve outcomes.

How the project is built

Three pieces fit together: data layer, analytics layer, presentation layer. Each can be developed and deployed independently.

01 Data layer SQL

PostgreSQL schema with 6 tables.

17 analytical queries answering business questions about patients, doctors, revenue, diagnoses.

Common pitfalls (integer division, name collisions in GROUP BY, lexicographic comparison) flagged and fixed.

02 Analytics layer Power BI

6 interactive dashboard pages (Demographics, Specialty, Diagnosis, Financial, Operations, Patient).

23 DAX measures: counts, ratios, period comparisons, classifications.

Sort-by columns for chronologically-ordered axes (Age Group, Day of Week, Month).

03 Presentation layer Vitalis Web App

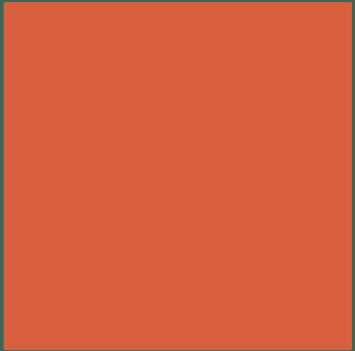
React + Vite + Tailwind + Recharts.

Mirror of the Power BI dashboards with cross-filtering, light/dark mode, LocalStorage persistence.

Three pages: Dashboard, Add Record, Settings — all built on the same dataset.

VITALIS

Summary



KEY TAKEAWAY

Healthcare data, transformed into action.

This project demonstrates how patient, financial, and operational data can be transformed into actionable insights using SQL, Power BI, and a modern web stack.

The analysis surfaces strong revenue performance in specific locations and specialties, operational inefficiencies (notably a 33% no-show rate), and the disproportionate impact of chronic disease and aging populations on healthcare costs.

Acting on these findings can improve patient outcomes, operational efficiency, and financial sustainability simultaneously — and the dashboards make those decisions easier to make.